INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Paclitaxel Days 1, 8 and 15 · relapsed or refractory small-cell lung cancer (SCLC)

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
28 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00621
Version 3

One infusion each week for three weeks, then one week without treatment. This 28-day cycle repeats while helping and tolerated.

Treatment stages

01 **Three weekly doses, then one week off**
Days 1, 8 and 15

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

SACTCHECK · CHAPTER 01

Understanding your treatment

Paclitaxel Days 1, 8 and 15 · relapsed or refractory small-cell lung cancer (SCLC) · National Cancer Control Programme (NCCP) 00621

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call the oncology contact number immediately for fever, shivers or sudden illness, severe or new breathing difficulty, chest pain, fainting, severe abdominal pain or persistent diarrhoea. Tell staff immediately about infusion symptoms. Report new numbness, weakness, painful skin changes, swelling or visual changes promptly; do not wait for the next cycle.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Your medicines and what they do

Treatment aims to control small-cell lung cancer after previous therapy and may help symptoms. The evidence is limited, so discuss likely benefit, burdens and alternatives with the team.

TAXANE CHEMOTHERAPY

Paclitaxel

Taxol (conventional paclitaxel)



Locks the cell division machinery

Cells use tiny structures called microtubules to separate their chromosomes when dividing. Paclitaxel stabilises these structures so they cannot rearrange normally, interrupting cell division. It can also affect nerves and other healthy tissues.

Microtubules form → Structures remain locked → Division is interrupted

What to watch for

Report tingling or numbness before the next weekly dose, especially if it affects walking or hand use. Tell the nurse immediately about flushing, chest or throat tightness, itching, dizziness or breathing changes during infusion.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Your lung treatment plan

Exact cancer setting

NCCP 00621 d: relapsed or refractorysmall-cell lung cancer. This use is outside the Irish licence and needs the hospital off-label approval and consent process. This is conventional paclitaxel monotherapy.

Course and visits

Paclitaxel is given on Days 1, 8 and 15 of each 28-day cycle. Day 22 is a planned week off, not a fourth infusion. Treatment continues while helping and tolerated; there is no fixed cycle limit.

What the evidence can tell us

A small uncontrolled study found some tumour responses to weekly paclitaxel in previously treated small-cell lung cancer (SCLC). The study used six weekly doses followed by two weeks without treatment, which differs from this NCCP schedule. It cannot establish a survival advantage over other treatment or supportive care.

Why this treatment may be chosen

Discuss possible benefit, alternatives and uncertainty for your own situation. The treatment aim is cancer control and symptom relief.

SACTCHECK · CHAPTER 02

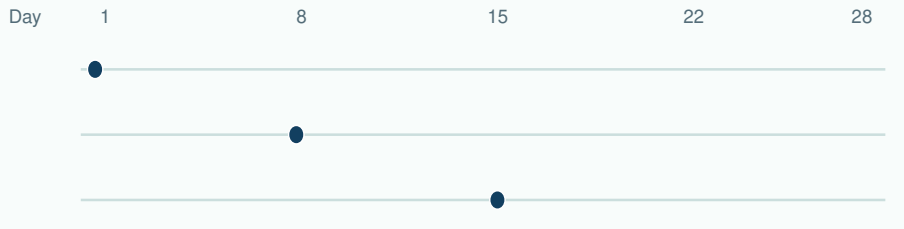
Your schedule, tests and appointments

Paclitaxel Days 1, 8 and 15 · relapsed or refractory small-cell lung cancer (SCLC) · NCCP 00621

Treatment pattern

Days 1, 8 and 15

28-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

DAYS 1, 8 AND 15

Three weekly doses, then one week off

Day1 | Day8 | Day15 | Weekoff

Paclitaxel is given on Days 1, 8 and 15 of each 28-day cycle. Day 22 is a planned week off, not a fourth infusion. Treatment continues while helping and tolerated; there is no fixed cycle limit.

The team confirms your actual prescription and appointments. This map explains the pattern and does not decide whether a dose can be given.

BASELINE → TREATMENT → FOLLOW-UP

Three treatment visits and an interval

This 28-day schedule has no Day 22 dose. No fixed scan interval or guaranteed recovery day.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00621 v3 · official regimen

THE WHOLE TREATMENT JOURNEY

Paclitaxel Days 1, 8 and 15 · relapsed or refractory small-cell lung cancer (SCLC): treatment and checks

Lung · relapsed/refractorysmall-cell lung cancer

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Day1	Day8	Day15	Days22–28	Nextcycle
Paclitaxel NCCP 00621	Confirm aim and off-label consent	Infusion	Infusion if appropriate	Infusion if appropriate	No Day 22 dose	Repeat if helping/ tolerated
Tests and symptoms NCCP 00621	full blood count (FBC), kidney, liver; nerves	Review before dose	Review before dose	Review before dose	Call if unwell	Review before dose
Premedication and review NCCP 00621	Confirm first-dose plan	Prescribed premedication	Later-dose plan	Later-dose plan	Continue symptom plan	Confirm appointments

What each check is for

1 Before each dose

Days 1, 8 and 15

Blood results and symptoms can change

Ask / act: Review before every dose and confirm premedication.

2 Fourthweek

Days 22–28

No scheduled infusion

Ask / act: Call if unwell; agree the next cycle and response-review plan.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Your practical treatment plan

STEP 1

Before the visit

Confirm the next appointment on Days 1, 8 or 15. There is no Day 22 dose. Blood results and symptom review are needed before every dose.

STEP 2

At the unit

Paclitaxel is given through a cannula or suitable existing intravenous (IV) line over about one hour. The appointment also includes checks, preparation, premedication and observation and is longer than the infusion itself. This regimen has no routine take-home pump.

STEP 3

At home

Keep the contact number, medicine list and symptom record available. Ask who will arrange your next blood test, scan and review. Sedating medicines may affect driving.

Checks through treatment

Your team arranges tests, reviews results and confirms each dose.

Blood tests and symptoms

Blood count, kidney and liver tests at baseline and before each treatment. Review symptoms, function, neuropathy and infusion reactions before every dose. Scans follow the consultant's individual plan.

Heart and breathing

Check observations during infusion, especially early in the infusion. Significant conduction problems require a specialist cardiac-monitoring plan. Routine serial echocardiograms are not imposed for this monotherapy.

Response review

The consultant agrees when scans and other disease checks are due. New or worsening symptoms may need assessment sooner. There is no automatic fixed scan interval in this regimen.

SACTCHECK · CHAPTER 03

Symptoms and supportive care

Paclitaxel Days 1, 8 and 15 · relapsed or refractory small-cell lung cancer (SCLC) · NCCP 00621

Days 1, 8 and 15

Day 1 → Day 28: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Paclitaxel Day 1	 **Day 1** intravenous (IV)
Paclitaxel Day 8	 **Day 8** IV
Paclitaxel Day 15	 **Day 15** IV
Nerves / muscle aches	 **Days 1–28** May persist or accumulate
Blood counts / energy	 **Days 1–28** Review before each dose; report illness any day

Three visits per cycle: Paclitaxel is given on Days 1, 8 and 15 of each 28-day cycle. Day 22 is a planned week off, not a fourth infusion. Treatment continues while helping and tolerated; there is no fixed cycle limit.

No guaranteed safe days: Symptoms can occur in the planned fourth-week interval too; call promptly if unwell.

> **Why these bands stay broad:** Bars show planned treatment and possible symptom patterns, not measured risk. Side effects can overlap and persist. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00621 v3 · official regimen ↗

There are no guaranteed "safe days". Act on fever, sudden illness or a new serious symptom whenever it occurs. Do not wait for the next appointment.

Between-treatment notes

Day 1: First visit of this cycle

Review symptoms, blood results and premedication before the infusion. Tell the nurse straight away about itching, flushing, throat or chest tightness, back pain, dizziness or breathing change. Check transport if premedication makes you drowsy and confirm the next weekly visit.

Day 2–7: Between weekly doses

Aches, fatigue, nausea or bowel symptoms may occur. Tingling may be mild at first but matters before the next dose. Record what symptoms stop you doing. Contact the team for new or worsening numbness, balance trouble, marked weakness or uncontrolled symptoms. Fever or sudden illness needs immediate contact.

Day 8: Second weekly review

This dose requires its own symptom and blood-test review. Describe the worst symptoms since the previous visit, not only how you feel today. Report sensory changes before the infusion and confirm your premedication instructions.

Day 9–14: Keep tracking function

Repeated treatment can increase nerve symptoms and fatigue. Check practical tasks such as buttons, writing and walking. Use the symptom passport to show changes. Seek early advice rather than waiting until numbness is severe.

Day 15: Third weekly review

The team decides whether treatment can proceed using the current assessment. Confirm your next review and the planned treatment-free fourth week.

Day 16–21: After the third visit

Symptoms may settle or continue. An interval between appointments does not rule out infection or a treatment complication. Use the urgent contact plan if unwell and bring your record to the next review.

Day 22: Planned week off

There is no Day 22 infusion in NCCP 00621. This interval does not guarantee that blood counts or symptoms have recovered. Continue your urgent contact plan and confirm the next cycle appointment.

Day 23–28: Complete the cycle record

Record symptoms and functional changes, including those that improve before the next appointment. Treatment continuation depends on benefit, toxicity and your priorities. Check the next cycle date and review plan. Do not treat a gap in visits as a guarantee of recovery.

Support around the medicines

1 Premedication

Premedication reduces infusion-reaction risk. The first-dose and later-dose plans may differ under the NCCP weekly pathway. The team must reconcile your exact product and reaction history; do not omit premedication yourself.

2 Nausea, bowel symptoms and pain

Use your individual anti-sickness and pain plan. Contact the team if symptoms are uncontrolled or you cannot drink. Antihistamines and other support medicines may cause drowsiness; arrange transport if advised.

3 Nerves and daily activities

Report tingling, numbness or weakness early, including effects on buttons, writing, walking or balance. Protect hands and feet and take care with hot surfaces if sensation changes.

4 Infection and blood counts

Each treatment needs its own blood and symptom review. A growth-factor injection may be prescribed after individual risk assessment; follow its separate instructions. Fever or sudden illness needs immediate contact on any day.

Practical support

Bring your medicine list and symptom record to every review.

Medicines and vaccines

Check new prescriptions, over-the-counter medicines, supplements and vaccine timing with the oncology pharmacist. Interactions and changes in organ function can alter the treatment plan.

Fertility and contraception

Discuss fertility preservation before treatment where relevant. Ask for written contraception and breastfeeding advice for your actual product and the period after treatment. Tell the team promptly about a possible pregnancy.

Support and priorities

Ask about symptom control, nutrition, rehabilitation, emotional support and help at home. Review how the treatment burden and possible benefit fit your priorities.

Questions worth asking

What benefit is realistic for me and what other options are available?

Treatment aims to control small-cell lung cancer after previous therapy and may help symptoms. The evidence is limited, so discuss likely benefit, burdens and alternatives with the team. A small uncontrolled study found some tumour responses to weekly paclitaxel in previously treated SCLC. The study used six weekly doses followed by two weeks without treatment, which differs from this NCCP schedule. It cannot establish a survival advantage over other treatment or supportive care.

Who arranges and reviews my blood tests and scans?

Blood count, kidney and liver tests at baseline and before each treatment. Review symptoms, function, neuropathy and infusion reactions before every dose. Scans follow the consultant's individual plan. Ask who arranges and reviews the results.

What premedication and anti-sickness plan should I follow?

Follow the written plan for your exact prescription. Report missed premedication before infusion; confirm prevention and rescue medicines with the pharmacist.

Which symptoms need an immediate call, and which number should I use?

Call the oncology contact number immediately for fever, shivers or sudden illness, severe or new breathing difficulty, chest pain, fainting, severe abdominal pain or persistent diarrhoea. Tell staff immediately about infusion symptoms. Report new numbness, weakness, painful skin changes, swelling or visual changes promptly; do not wait for the next cycle. Keep the unit's daytime and out-of-hours numbers on your treatment card.

> **Try explaining the plan back:** Can you explain the treatment aim, your next visit, the blood-test plan, premedication and when to call for urgent help?

Treatment scope · NCCP 00621 NCCP 00621d: small-cell lung cancer that has returned or has not responded to previous treatment. This use is outside the Irish licence and requires the hospital off-label approval and consent process. This guide is for conventional paclitaxel alone.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Paclitaxel · Days 1, 8 and 15 of 28 · reviewed supportive care

Active medicines: Paclitaxel

Emetogenic risk: Low

Medicine	Dose / route	When
Ondansetron	8 mg orally	On treatment day Review QT, constipation and hepatic dosing; reconcile any steroid already prescribed
Metoclopramide	10 mg orally up to three times daily as needed	Subsequent days: duration to be confirmed Assess contraindications, renal function and interactions

Additional phase-specific support

- First paclitaxel dose: dexamethasone 8 mg intravenous (IV), chlorphenamine 10 mg IV and famotidine 20 mg IV, each 30 minutes before treatment (NCCP suggested table).
- From Day 8 onward: chlorphenamine 10 mg IV and famotidine 20 mg IV 30 minutes before treatment. Routine dexamethasone may be omitted after the first dose; prior reaction or increased risk requires consultant review.

- Famotidine may be omitted under the consultant/local plan if appropriate. A prior reaction requires a separate rechallenge assessment; do not reuse the standard draft automatically.
- First-dose NCCP suggested premedication: dexamethasone 8 mg IV, chlorphenamine 10 mg IV and famotidine 20 mg IV 30 minutes before paclitaxel.
- From Day 8, dexamethasone may be omitted in the source suggested table; add or modify for increased risk or previous reaction under consultant guidance. Famotidine omission is also a consultant/local-policy decision requiring close observation.
- The conventional paclitaxel product information has a different generic premedication regimen. Reconcile the exact product, weekly NCCP pathway and local policy; never self-omit premedication.
- Low emetic risk; keep the antiemetic plan separate from reaction premedication. Consider analgesia for myalgia/arthralgia and early action on neuropathy.
- Confirm first-dose versus later-dose paclitaxel premedication. No Day 22 treatment in 00621.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Your anatomical side-effect map

Paclitaxel Days 1, 8 and 15 · relapsed or refractory small-cell lung cancer (SCLC)

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Hands / feet

Paclitaxel

Report numbness, weakness or changes in walking and hand use before the next dose.

3 Hair / scalp

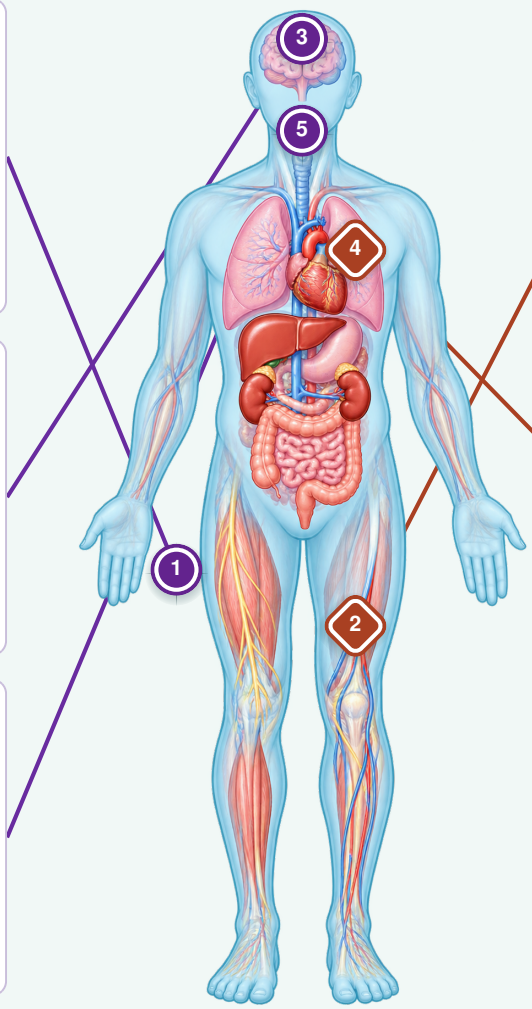
Paclitaxel

Hair loss can occur; ask about support before treatment.

5 Mouth

Paclitaxel

Report painful ulcers, difficulty drinking or poor intake.



2 Blood / infection

Paclitaxel

Fever, shivers or sudden illness need immediate contact.

4 Breathing

Paclitaxel

New or worsening breathlessness or cough needs urgent assessment.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Sources and further information

Paclitaxel Days 1, 8 and 15 · relapsed or refractory small-cell lung cancer (SCLC) · National Cancer Control Programme (NCCP) 00621

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00621 v3 · official regimen**
   https://healthservice.hse.ie/documents/6854/621_V3_Paclitaxel_80_zanF78k.pdf
2. **HPRA conventional paclitaxel SmPC; 9 January 2026**
   https://assets.hpra.ie/products/Human/27560/Licence_PA2315-115-001_09012026104101.pdf
3. **Yamamoto et al., Anticancer Research 2006; relapsed / refractory SCLC**
   https://pubmed.ncbi.nlm.nih.gov/16739353/
4. **NCCP NCIS medical-oncology antiemetics V 8**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
5. **NCCP antiemetic guidance; reconcile with regimen and individual prescription**
   https://healthservice.hse.ie/documents/7152/Classification_Document_for_Systemic_Anti-_Cancer_Therapy_systemic anticancer therapy (SACT)_Induced_Nausea__pWgDCrm.pdf